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Informal payments and the illusion of free care: a perspective from Somalia's fragile health system

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Abstract

Universal Health Coverage (UHC) promises access to care without financial hardship, yet in many low- and middle-income settings this ideal is undermined by pervasive informal payments. This perspective introduces the concept of the “illusion of free care” to explain how official fee-exemption policies coexist with hidden, off-the-record charges that reshape access and equity. Drawing on Somalia as a critical case of health system fragility, we argue that informal payments are not isolated acts of corruption but predictable responses to chronic underfunding, weak governance, and workforce precarity. We conceptualize this dynamic as a four-stage process linking policy promises, systemic constraints, the normalization of informal payments, and inequitable patient outcomes. These practices disproportionately exclude the poorest, erode trust, and stratify care along socioeconomic and gender lines. Conventional policy responses, including anti-corruption measures and nominally free care policies, fail to address underlying structural drivers. Achieving equitable UHC requires shifting from rhetorical commitments to systemic reforms in financing, workforce protection, and accountability.

Keywords Informal payments, Universal health coverage, Health equity, Fragile health systems, Out-of-pocket expenditure, Somalia

Introduction

The global push toward Universal Health Coverage (UHC) promises that everyone can access needed services without financial hardship, often translated politically and programmatically into slogans of “free healthcare” at the point of use [1, 2]. Yet across many low- and middle-income countries, especially in

sub-Saharan Africa, this promise collides with persistent out-of-pocket spending and covert fees that quietly reintroduce payment into supposedly free care [3, 4]. Informal payments off-the-record, unofficial monetary or in-kind transfers made directly to health workers or facilities, whether “voluntary” gifts or coerced bribes have become a ubiquitous feature of healthcare in such settings [4, 5]. They range from small cash sums to obtain drugs or avoid queues, to larger, sometimes extortionary, demands for services that policy declares free [4, 5]. Far from marginal irregularities, these payments often constitute a substantial share of health financing, reshape who gets treated and how, and systematically disadvantage the poorest [4, 6]. Somalia illustrates this dynamic in its starkest form [7, 8]. A highly privatized delivery

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landscape, extremely low public health spending, heavy donor dependence, and weak regulation have produced a fragmented system where out-of-pocket payments dominate and sustainability and equity are in question [8, 9]. In such a context, informal payments are not surprising deviations but predictable adaptations to chronic underfunding, parallel aid systems, and absent accountability [4, 10]. This perspective advances the concept of the “illusion of free care”: a policy narrative of no-fee services masking a parallel, informal economy of access. Using Somalia as an archetype of structural fragility, it argues that confronting UHC’s equity failures requires making these hidden payment regimes visible and analytically central, rather than treating them as peripheral corruption problems.

Conceptual framing: informality, fragility, and equity

Conventional anti-corruption narratives cast informal payments as deviant acts by unethical providers or “bad apples” [11]. Yet comparative and institutional analyses show that informal payments arise where formal rules, resource allocation, and social norms are systematically misaligned, not where individual morality alone fails [12, 13]. Informal payments thus signal deeper health system fragility: chronic underfunding, unreliable supplies, weak oversight, and low trust in public institutions that together erode the credibility of “free care” guarantees [14, 15].

Fragile and conflict-affected health systems are defined by unstable governance, volatile financing, and heavy dependence on external actors, conditions that constrain absorptive and adaptive capacity and entrench parallel arrangements for access to care [16, 17]. In such environments, informality becomes a mode of governance: rules are negotiated at the facility or provider level, transparency is low, and enforcement is selective or absent [4, 14]. Informal payments, often framed as “gifts” or local problem-solving, are better understood as embedded practices that compensate for institutional gaps, while simultaneously reproducing structural inequalities in who can pay, negotiate, or refuse [15, 18]. The “Illusion of Free Care” framework conceptualizes this dynamic as a four-step system process. First, policy promises of free or subsidized services create an entitlement narrative, often central to UHC and fee-exemption reforms [4, 19]. Second, severe system constraints low and unstable public expenditure, health worker precarity, and absent accountability undermine the state’s capacity to honor these entitlements [14, 20]. Third, providers and facilities respond through the emergence of informal payments to fill operational gaps, supplement income, or manage demand, normalizing off-the-record exchanges as part of routine care [14, 18]. Finally, these practices

yield patient-level inequitable outcomes: delayed or forgone care, differential treatment by ability to pay, and disproportionate hardship for poorer and marginalized groups [15, 21]. By tracing this sequence, the Illusion of Free Care framework reveals hidden layers of exclusion that persist beneath UHC rhetoric, recasting informal payments from marginal corruption to a core expression of structural fragility and inequality in settings such as Somalia.

Somalia as a critical case study

Somalia’s health system exemplifies the “illusion of free care,” where policy promises of universal access are systematically undermined by structural deficits and the pervasive reality of informal payments [8, 9]. Chronic underfunding is foundational: government health expenditure remains among the lowest globally, with less than 10% of total health spending coming from public sources [8]. Instead, Somalia relies heavily on donor funding and direct household out-of-pocket (OOP) payments, which account for nearly 45% of total health expenditure—one of the highest rates worldwide [8, 10]. This fiscal fragility is compounded by weak regulatory systems; oversight is minimal, accountability mechanisms are fragmented, and enforcement of formal rules is inconsistent or absent [8, 9]. In this vacuum, the private sector dominates service provision, especially in urban areas, operating largely unregulated and on a fee-for-service basis that exposes patients to unpredictable costs for consultations, drugs, diagnostics, and even basic maternal care [10, 22]. Within this context, informal payments emerge as both coping mechanisms and exploitative practices [23]. On one hand, health workers facing low or irregular salaries and resource shortages solicit informal payments to supplement income or procure essential supplies [1, 24]. These payments may be framed as “gifts” or necessary contributions to ensure drug availability or expedite services [23, 24]. For example, patients frequently pay under-the-table for medicines that should be provided free at public facilities or to access diagnostics and maternal care services such as deliveries or caesarean Section [23, 25]. Such practices are often rationalized by providers as survival strategies in a system that fails to meet operational needs [1, 24]. However, these same dynamics create fertile ground for exploitative behaviors linked to power asymmetries [23, 24]. Providers with greater authority or control over scarce resources can extract higher informal fees or prioritize those able to pay more a pattern observed in both public and private facilities [22, 26]. The blurred line between coping and exploitation is further complicated by social norms around gift-giving and gratitude but ultimately reflects deeper governance gaps [23, 24]. The consequences are starkly inequitable [10]. Poor households face catastrophic expenditures or are forced

to delay or forego care altogether; inability to pay often results in lower-quality treatment or outright exclusion from essential services [8, 10, 25]. Internally displaced persons (IDPs), women, and children are disproportionately affected: IDPs lack social networks for negotiation; women encounter additional barriers in accessing maternal care due to both financial constraints and gendered power relations; children's access to immunization or emergency care is similarly compromised [6, 25]. These outcomes reinforce structural inequalities those least able to pay bear the greatest burden of unmet health needs [6, 8].

Equity implications: beyond financial barriers

Informal payments in fragile systems like Somalia do not only create extra costs; they reorder who is seen, how they are treated, and whose lives are considered expendable [14, 15]. They transform a nominally universal system into a stratified one where ability to pay, gender, and social status quietly determine health outcomes [15, 27].

Catastrophic, hidden costs and forgone care

Informal fees add a hidden financial layer on top of already high out-of-pocket spending, often constituting a substantial share of total payments and pushing households toward catastrophic expenditure and poverty [14, 15]. Even “small” informal charges for immunization, family planning, deliveries or certificates have been shown to exclude the poorest women and drive them to unlicensed or lower-quality providers [14, 18]. Inability to pay is directly linked to delayed or forgone care and lower-quality treatment, particularly for the poor [14, 15].

Trust, social contract, and moral dilemmas

When patients are asked to pay unofficially for services that are supposed to be free, they experience the health system as arbitrary and predatory, eroding confidence in public institutions and in the promise of UHC [18, 20]. This signals a breakdown of the social contract: the state's commitment to protect citizens' health is replaced by opaque, negotiable entitlements mediated by cash and connections [12, 28]. Providers themselves are often caught in moral dilemmas using informal payments to keep facilities functioning or supplement inadequate pay, while knowing these practices exclude the most vulnerable and damage professional ethics [11, 20, 29].

Gender, vulnerability, and layered exclusion

Evidence from maternal and family planning services shows that informal payments are routine and particularly exclusionary for women, who may forego needed care or accept lower-quality alternatives when unable to pay [14, 18]. Gendered power relations and

poverty intersect so that women, especially those with low income or in precarious situations, experience both financial and relational coercion in encounters with providers [14, 18, 27]. Poor households, IDP-like populations, and marginalized groups face higher risks of being unattended, turned away, or pushed into debt when informal fees are demanded [14, 15, 28]. Informal payments therefore crystallize systemic inequity: they convert the illusion of free care into a regime of selective inclusion in which those already disadvantaged by poverty, displacement, or gender bear the greatest burden of unmet need, mistrust, and moral harm [12, 15, 30].

Why conventional solutions fail

Anti-corruption strategies typically focus on detecting and punishing individual misconduct, assuming informal payments are primarily a problem of rule-breaking actors [4, 15]. Yet evidence from Nigeria and wider sub-Saharan Africa shows that informal payments are enabled by systemic drivers financing shortfalls, unclear fee policies, weak oversight, and practical norms not just individual greed [4, 14, 15]. Narrow integrity approaches therefore treat symptoms while leaving underlying incentives and constraints untouched [4, 15]. User fee removal and “free care” policies without systemic reform replicate this misdiagnosis [4, 5]. Reviews across SSA show that even under insurance or exemption schemes, patients continue to pay formal and informal fees because facilities lack operating funds, drug supplies, and clear implementation rules [4, 5]. In Enugu, primary health centres, cut off from operational funding, were effectively instructed to “generate their own revenue,” institutionalizing informal charges despite nominally free or subsidized care [14]. Without addressing financing gaps, facility autonomy becomes a license to shift costs onto patients [4, 14]. Donor-driven vertical interventions also fail to dislodge informal payments where they leave core system functions untouched [5, 15]. In Somalia, donor aid and projects plug specific programmatic gaps but cannot substitute for sustainable pooled funding, robust public regulation, or an adequately paid, equitably distributed workforce [9, 31]. Vertical programmes may improve selected indicators while everyday encounters remain governed by user fees, OOP spending, and discretionary charging [4, 5]. Across these responses, informal payments persist because the structural conditions that produce them chronic underfunding, workforce insecurity (low/irregular salaries, precarious contracts), and governance weaknesses (fragmented regulation, poor supervision, opaque revenue practices) remain intact [14, 15]. Tackling informal payments, and the equity failures they encode, thus requires moving beyond technocratic fixes toward a systemic transformation agenda: sustainable financing, accountable governance, and labour

conditions that make “free care” more than an illusion [4, 5, 15].

Reframing solutions: from informality to system reform

Forward-looking policy responses must replace the illusion of free care with financing and governance arrangements that make entitlements real, especially for those most exposed to informal payments [15]. Sustainable health financing in Somalia cannot rely on volatile aid and out-of-pocket (OOP) spending; experience from African settings points to expanding prepaid, pooled financing through progressive taxation and mandatory insurance combined with explicit protection for the poorest to reduce catastrophic and impoverishing OOP payments [15, 32]. External funds should be harmonized with domestic budgets and targeted to primary care in underserved and displacement-affected areas, rather than fragmented vertical projects [32]. Health workforce protection is central to dismantling incentives for informal charging [15, 20]. Evidence from Tanzania and Nigeria shows that low and irregular pay, lack of benefits, and precarious contracts are key supply-side drivers of informal payments [15, 20]. A realistic agenda would prioritize: (1) ensuring timely, predictable salaries for frontline staff; (2) formalizing community and lower-cadre workers with contracts and basic labour protections; and (3) using targeted incentives in hardship and IDP-hosting areas so that coping does not depend on extracting fees from the poorest [15, 20]. Governance and accountability mechanisms must directly tackle the governance gaps that allow informal markets to flourish [15, 20]. Tools that diagnose governance weaknesses in transparency, accountability, participation, integrity, and capacity can be adapted to fragile settings to identify concrete reforms such as defining and enforcing quality and waiting-time guarantees and penalizing all forms of informal payments, not only proven bribes [33].

Transparency interventions such as publicly posted fee schedules, complaint hotlines, and safe whistle-blower systems have reduced corruption and counterfeit drugs elsewhere, but need clear equity goals and sustainable design to endure [15, 34]. In Somalia, these measures should be co-designed with women, IDPs, and marginalized groups and linked to real sanctioning and remediation power [15, 35]. Prioritizing pro-poor targeting in financing, workforce deployment, and accountability mechanisms aligns policy with those currently excluded, shifting from an illusion of free care to a progressively universal system that constrains informal payments at their structural roots [1, 32].

Broader lessons for fragile settings

Somalia's heavy reliance on out-of-pocket payments, limited government financing, and fragmented, donor-dependent provision makes it an archetype of how fragile systems generate the illusion of free care while shifting costs onto households [9, 10]. Extremely high OOP shares, low UHC scores, and widespread unmet need position Somalia at the extreme end of patterns also seen in other conflict-affected and low-income settings [7, 10].

Generalizable lessons on informal payments and equity

Evidence from Nigeria, Tanzania, Kenya, Iran, Ukraine and EU countries shows that informal payments emerge wherever public spending is low, governance weak, and access mediated by discretion rather than enforceable rights [4, 15]. In Enugu, “free” maternal and child health care is routinely accompanied by informal fees that exclude the poorest women [14] while in Kenya and SSA more broadly, insured clients still pay under-the-table for covered services, reflecting implementation gaps and practical norms [4]. In Europe, lower public health expenditure correlates with higher informal payment prevalence, though institutional quality also matters [4].

From Somali experience to global UHC policy

Somalia therefore functions as a sentinel case: its extreme OOP dependence, fragmented governance, and inequitable access make visible dynamics that operate more subtly elsewhere [7, 9, 10]. The lesson is not that Somalia is exceptional, but that informal payments are a predictable outcome of underfinanced, weakly regulated systems whether in FCAS, SSA, MENA, Southeast Asia or parts of Europe [1, 4, 32]. For global health and UHC agendas, this implies that tackling informal payments is inseparable from securing sustainable pooled financing, protecting informal workers and the poor, and strengthening accountability; otherwise, “free care” commitments will continue to mask structurally produced inequity [1, 15, 32]. Somalia's experience signals that without structural reform, informal payments are not aberrations but systemic by-products, demanding that UHC reforms confront financing and governance foundations rather than only their visible symptoms [7, 10].

Conclusion

In fragile health systems like Somalia's, the promise of “free care” is often a mirage one that conceals deep and persistent inequities produced by informal payments. When public financing is insufficient and governance fragmented, as in Somalia, health workers and facilities turn to informal charges to fill operational gaps, shifting the financial burden onto households least able to bear it. This dynamic is not unique: across sub-Saharan Africa and other low-resource settings, informal payments

routinely emerge wherever system capacity is lacking, undermining both access and trust in healthcare. The evidence is clear that such hidden costs disproportionately exclude the poorest and most vulnerable, driving them away from essential services or forcing them into debt and hardship.

Superficial solutions such as isolated anti-corruption campaigns or piecemeal fee exemptions have repeatedly failed to address the root causes of informal payments. Instead, research underscores the need for structural reform: sustainable pooled financing, robust accountability mechanisms, fair provider remuneration, and genuine community oversight are essential to dismantle the incentives that perpetuate these practices. Without these foundational changes, “free care” policies will continue to mask rather than resolve inequity.

The urgency for equity-centered transformation in fragile settings cannot be overstated. As Somalia's experience reveals, justice in health is not achieved by rhetoric or policy declarations alone but by confronting the systemic deficits that breed exclusion. True universal health coverage demands more than the illusion of free care it requires a commitment to building systems where access is real, rights are protected, and no one is left behind.

Author contributions

IAK conceptualized the manuscript, developed the central “illusion of free care” framework, and led the overall design, analysis, and writing as first and corresponding author. YAH drafted the initial full version of the manuscript, contributed to integration of the conceptual framework, and coordinated revisions. ANA and NAD contributed to literature synthesis, strengthened sections on health system fragility and informal payment dynamics, and enhanced conceptual clarity. AMA supported the development of equity and governance arguments and assisted in manuscript editing and refinement. SGB provided senior supervision and mentorship, guided the conceptual direction, and critically reviewed the manuscript to strengthen policy relevance, coherence, and intellectual rigor. All authors reviewed and approved the final version of the manuscript and agree to be accountable for the accuracy and integrity of the work.

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Data availability

No datasets were generated or analysed during the current study.

Declarations

Ethics approval and consent to participate

This article is a conceptual perspective that synthesizes and critically interprets existing peer reviewed literature to advance theoretical understanding and policy-relevant insights on the topic. It does not present new empirical data or conduct a formal systematic or narrative review.

Consent for publication

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Competing interests

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